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BEYOND THE "TWO PSYCHIATRIES": JACK PRESSMAN'S *LAST RESORT* AND THE HISTORY OF TWENTIETH-CENTURY AMERICAN PSYCHIATRY

Jack Pressman and the Future of the History of Psychiatry

JOHN C. BURNHAM

I would like to attempt to forecast the direction in which historians of psychiatry will be headed in the immediate future, focusing first on the circumstances and then on the narrative of the history of psychiatry. My take is similar to, and yet a little different from, that of Joel Braslow in this symposium.

Prediction

Reading the future is of course a difficult, if not an impossible, task. I am fond of quoting the wisdom, variously attributed, that predicting is very difficult, especially concerning the future.[1] We are all forced, throughout our lives, for our own guidance, to project the past and present into the future, and on this occasion I particularly want to suggest how Jack Pressman's book affords a glance into what lies ahead for historians of psychiatry in the next few years.

In trying to read what is coming, I shall not go through all of the philosophical contingencies that Nicholas Rescher describes in his comprehensive new book on predicting the future. Both technical analysis and common sense, however, suggest that the best possible information on current trends, along with past experience, yields the best predictors.[2]

1. John C. Burnham, "The Past of the Future of Medicine," *Bull. Hist. Med.*, 1993, *67:* 1–27.
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A dozen years ago, Nancy Tomes reviewed the then-most-recent publications in the history of psychiatry. She found that already a "'post-Foucaultian' generation" had appeared, and that cutting-edge historians were no longer obsessed with "the 'march of progress' versus the 'great confinement,'" and the "treatment-incarceration dichotomy."[3] Indeed, she was writing an epitaph for the social control writers who conceived of the history of psychiatry as the history of psychiatrists' imposing their "power" on patients and society.[4]

The Changing Presents of Presentism

Since Tomes wrote, it has become more obvious how the writing of the history of psychiatry has been further affected by forces in the contemporary world. Therefore, like so many others attempting to explain changes in historical writing, I wish to revert to the tired but valid cliché that all historians write to a variety of presents, even as the better ones try to avoid the worst consequences of Whig history. For it is not possible to discuss the historiography of psychiatry without mentioning the impact of recent forces upon the specialty itself.

The first force is the circumstance that, in the United States, the formerly large and flourishing specialty of psychiatry is being reshaped (if not substantially destroyed) by external socioeconomic conditions, acting chiefly through managed care. A second current force is the dominance within psychiatry of a biological and pharmacological approach to mental illnesses.

Not even the most antiquarian historian, it seems to me, can ignore the remarkable erosion of the economic base of the specialty of psychiatry. Do we have, in the history of medicine, any precedents for a specialty that was so damaged by outside economic forces? Some small fields dependent upon a particular instrument or technology might come to mind, such as the late-nineteenth-century practice of electrotherapeutics. But that development, one might argue, reflected changes in medical science, not an outside economic force.[5] We have many other examples

3. Nancy Tomes, "The Anatomy of Madness: New Directions in the History of Psychiatry," *Soc. Stud. Sci.*, 1987, *17:* 358–70, quotation on pp. 358–59.

4. It is of course only fair to note that some further work in this tradition has won recognition because of the sophistication and ability of the particular author, exemplified especially by Elizabeth Lunbeck, *The Psychiatric Persuasion: Knowledge, Gender, and Power in Modern America* (Princeton: Princeton University Press, 1994).

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of fields destroyed by new knowledge. At almost exactly the same time that electrotherapeutics was fading, for instance, the germ theory of disease destroyed the field of pharmacognosy. But the specialty of pharmacognosy had been limited largely to teachers of the subject in medical schools, and in other ways it does not offer an apt comparison to what is going on in psychiatry now.[6]

Perhaps the closest parallel of which we might all think was midwifery. By the mid-twentieth century, specialist historians like Harold Speert had written midwives out of the history of obstetrics. [7] But in this case, still another external force, modern feminism, quite transparently brought midwives back to life in history as part of a presentist campaign to revive birth attendance by midwives.[8] Will much of psychiatry likewise be forgotten until therapeutic focuses change again?

The question of the focus of therapy is complicated because of the problem to which I have alluded: much of what has been written in the history of psychiatry in the last half century was determined by a present that historians rather generally assumed. The formerly dominant present—until biological and pharmacological approaches became dominant—was one in which psychosocial etiologies and psychotherapies constituted the most exciting part of psychiatry. It was a time, also, as Gerald Grob has pointed out in rich detail, when hegemonic Americans turned away from the very ill patients and concentrated psychiatric

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1988, *43:* 64–82) found that the X ray effectively drove electrotherapeutics out of regular medicine. Or see Norman Gevitz, "Autonomous Profession or Medical Specialty? The Stomatological Movement and American Dentistry," *Bull. Hist. Med.*, 1988, *62:* 407–28.

6. Pharmacognosy is dealt with to some extent in John C. Burnham, *Jelliffe: American Psychoanalyst and Physician* (Chicago: University of Chicago Press, 1983). Perhaps one could cite the incidents in which various forms of psychotherapy fell out of favor in Germany and Russia for political—and therefore economic—reasons in the period between the two world wars. Local historians of the specialty did, it is true, devise temporary accounts to explain and justify those excisions, but within a generation or two, even those histories were back into the world mainstream. See, for example, Geoffrey Cocks, *Psychotherapy in the Third Reich: The Göring Institute,* 2d ed. (New Brunswick, N.J.: Transaction Publishers, 1997), which has some recent bibliographic citations added; and Alexander Etkind, *Eros of the Impossible: The History of Psychoanalysis in Russia,* trans. Noah and Maria Rubins (Boulder, Colo.: Westview Press, 1997).

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Part of the victory of psychosocial psychiatry involved the capture of the mainstream narrative of the history of psychiatry. But that revised, psychosocial mainstream narrative preserved elements from an earlier standard narrative.[10]

The Original Standard Narrative

What happened was that by the early years of the twentieth century, psychiatric specialists had constructed a historical narrative based substantially upon changes in internal medicine that had been under way for some time. The narrative had two simultaneous aspects: on the one hand, medical science and the theory of mental illness, or psychopathology; and on the other hand, management and therapy. Management and therapy, in turn, were often conflated and confused with each other. Emil Kraepelin, for example, in his 1917 classic, *One Hundred Years of Psychiatry*, emphasized that the specialists offered both a systematic (as opposed to an empirical) approach to mental illness, and a sympathetic (as opposed to a punitive) approach to the patient. And for scientific support, he could already cite not only the discovery that paresis was a tertiary syphilis, but a whole panopoly of inferences based on the dawn of the physiological age in medicine just before World War I.[11]

But Kraepelin and others justified what they were doing by criticizing their predecessors—which made the history of psychiatry very important in justifying the existence of the specialty. He summarized the conditions that existed before psychiatric specialists came on the scene: "negligent and brutal treatment of the insane; improper living conditions and inadequate medical care; beclouded and false notions concerning the nature and cause of insanity; senseless, haphazard, and at times harmful therapeutic measures which aggravated the plight of those afflicted by mental illness."[12]

9. See, among his many publications, Gerald N. Grob, *From Asylum to Community: Mental Health Policy in Modern America* (Princeton: Princeton University Press, 1991).

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The Rise and Fall of the Dynamic and Environmentalist Narrative

The next generation of historians of psychiatry appropriated the same negative narrative, but now they added to it both a positive element and a new negative one. The positive element was an emphasis on psychological and environmental factors in both the etiology and treatment of mental illnesses, typically featuring Sigmund Freud as a point of departure. The new negative was excoriating the emphasis that previous writers had put on diagnosis and classification and—at least implicitly—hereditary factors and institutionalization. This narrative retained some of the positive aspects of the movement to hospitalize the ill, but in this history, what had started out to be humanitarianism, with so-called moral treatment, turned into neglect or abuse. There were any number of these new narratives written by dynamic psychiatrists, the best known being those of Gregory Zilboorg and George W. Henry, and of Franz Alexander and Sheldon T. Selesnick.[13]

For some time, institutional psychiatrists who emphasized management, and also physicians who advocated various somatic therapies, tried to set up competing histories that would feature the hospital tradition, or neuropathology and various shock and other treatments in the physiological tradition. But after the mid-1950s, in histories of psychiatry, the hospital tradition was obscured in the rush to deinstitutionalize patients. Finally, enthusiasts of the new biological and outpatient emphases in the specialty used psychopharmacology to discredit somatic treatments as well as psychotherapy—as in a very recent general history of psychiatry by Edward Shorter.[14] Once again, historical writing changed as the specialty changed.

The Fragmentation of the Narrative

We are fortunate in having two landmark books with which to measure what happened in the recent past of the historiography of psychiatry. The first book, edited by George Mora and Jeanne Brand and published

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in 1970, represented the continuing presence of practicing psychiatrists in the history-of-psychiatry enterprise; but at the same time, nonpsychiatrist historians, chiefly medical historians, contributed. In addition, underlining the environmental lens of this group, a number of social scientists were included.[15]

In 1994, Mark S. Micale and Roy Porter edited a quite different collection, one designed to be less overtly prescriptive (about how to write history) and more historiographic. There were still a number of practitioners who contributed, but this time the main burden fell on professional historians—a group who had now moved into the field in large numbers. And with the professional historians, the well-established narrative that still undergirded the contributions to the 1970 volume had fallen apart: "it is a basic contention of this book," wrote the editors, "that there are many separate *histories*"—not a single history of psychiatry.[16] These writers underlined Tomes's perception of what she called "uncertainty and complexity."[17] "We believe," they continued, "that in no branch of history of science or medicine has there been less interpretive consensus."[18]

What had happened, particularly, was that the professional historians and the practicing psychiatrists who kept up with them had brought still one more present into the history of psychiatry. This has generally gone under the heading of postmodernism. As was typical of this spasm of the 1980s, writers introduced romantic individualism, and they particularly wrote negatively about any institution—thus continuing the attempts of the social control school to discredit the asylum system and the asylum physicians.

Not all historians were postmodern in their orientation. But all of them together over a period of years did bring in a new emphasis on the patient, at the very least through the doctor-patient relationship—a historiography on which Joel Braslow is focusing in this symposium.

Finally: Specialty History and Eclecticism

By the 1990s, it was obvious that an older stream of history by and about psychiatric specialists was continuing to prosper. This was the stream of the history of a specialty. In the journal *History of Psychiatry* (which began

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in 1990), internal, or "conceptual," history was still unashamedly conspicuous, along with institutional and some social history.[19] This flourishing history of a specialty reemphasized the tie of the history of psychiatry to medical history per se, in which specialty histories constituted an important and flourishing segment—whether in surgery, neurology, pediatrics, or any other branch of medicine in which an emphasis on specialized knowledge kept a place open for physicians to write medical history.

Other kinds of historians of psychiatry, following a trend in more general history, strikingly embraced an even more inclusive eclecticism, with an empiricism reinforced by the *Annales* school.[20] It is this eclectic approach, applied to the mentally ill and their physicians, that is so striking in the 1994 book edited by Micale and Porter.

The Future: Dropping the Negatives

Now, several years later, in the work of Jack Pressman (and, if I might also mention a book by our collaborator in this symposium, Joel Braslow), it is possible to see even more clearly how the new history of psychiatry will probably develop.[21] Pressman's book, *Last Resort,* is a particularly transparent model for the future.

Last Resort was long in the making. I happen to have had the privilege of reading one of the middle drafts. The published book is very different from what I saw then, and it is the differences that suggest to me what the future may hold for the history of psychiatry. Basically what changed was that most of the postmodern formulations dropped out, and in their place appeared a really interesting new trend: Pressman's history of psychiatry interacts with and speaks to the history of medicine generally—the same quality that Joel Braslow exemplifies and talks about.

19. The authors were often sophisticated, but they did not reflect a postmodern demedicalization of the history of psychiatry or, for example, the decline of standard thresholds in physiologically oriented medicine—a startling departure from standardization in the medical "gaze" that paralleled the romantic individualism of the deinstitutionalizers.

20. See, e.g., Georg G. Iggers's summary, *Historiography in the Twentieth Century: From Scientific Objectivity to the Postmodern Challenge* (Hanover, N.H.: Wesleyan University Press, 1997), just one of many signs of the new eclectic approach that characterized the work of most thoughtful historians. Some historians retreating from the 1980s now use the oxymoronic phrase, "postmodern realism."

21. Joel Braslow, *Mental Ills and Bodily Cures: Psychiatric Treatment in the First Half of the Twentieth Century* (Berkeley: University of California Press, 1997).

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in 1990), internal, or "conceptual," history was still unashamedly conspicuous, along with institutional and some social history.[19] This flourishing history of a specialty reemphasized the tie of the history of psychiatry to medical history per se, in which specialty histories constituted an important and flourishing segment—whether in surgery, neurology, pediatrics, or any other branch of medicine in which an emphasis on specialized knowledge kept a place open for physicians to write medical history.

Other kinds of historians of psychiatry, following a trend in more general history, strikingly embraced an even more inclusive eclecticism, with an empiricism reinforced by the *Annales* school.[20] It is this eclectic approach, applied to the mentally ill and their physicians, that is so striking in the 1994 book edited by Micale and Porter.

The Future: Dropping the Negatives

Now, several years later, in the work of Jack Pressman (and, if I might also mention a book by our collaborator in this symposium, Joel Braslow), it is possible to see even more clearly how the new history of psychiatry will probably develop.[21] Pressman's book, *Last Resort,* is a particularly transparent model for the future.

Last Resort was long in the making. I happen to have had the privilege of reading one of the middle drafts. The published book is very different from what I saw then, and it is the differences that suggest to me what the future may hold for the history of psychiatry. Basically what changed was that most of the postmodern formulations dropped out, and in their place appeared a really interesting new trend: Pressman's history of psychiatry interacts with and speaks to the history of medicine generally—the same quality that Joel Braslow exemplifies and talks about.

19. The authors were often sophisticated, but they did not reflect a postmodern demedicalization of the history of psychiatry or, for example, the decline of standard thresholds in physiologically oriented medicine—a startling departure from standardization in the medical "gaze" that paralleled the romantic individualism of the deinstitutionalizers.

20. See, e.g., Georg G. Iggers's summary, *Historiography in the Twentieth Century: From Scientific Objectivity to the Postmodern Challenge* (Hanover, N.H.: Wesleyan University Press, 1997), just one of many signs of the new eclectic approach that characterized the work of most thoughtful historians. Some historians retreating from the 1980s now use the oxymoronic phrase, "postmodern realism."

21. Joel Braslow, *Mental Ills and Bodily Cures: Psychiatric Treatment in the First Half of the Twentieth Century* (Berkeley: University of California Press, 1997).

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But from a historiographic point of view, Pressman's surge into the future is most striking because he dropped the negative tone and deconstructionist stance. Furthermore, he dropped not only the postmodernists' nihilism but also the traditional negatives in the history of psychiatry—first of Kraepelin's generation, and then of the dynamic and community psychiatrists. It may be that Pressman as an especially well-trained historian was more successful than his predecessors in escaping Whiggish presentism. Regardless, the disappearance of the negatives represents a striking change.[22]

Some of the irony, which was never an exclusive possession of postmodernists, remains in *Last Resort*. But Pressman's positive unraveling of complexity, using a variety of types of insight and approach, produced a constructive, thought-provoking examination of physicians, patients, and society trying to deal with and understand mental illnesses. Sometimes even the most seriously mentally ill are now viewed in an intellectual environment heavy with genetics and biochemistry in which, as Ellen Dwyer suggests in this symposium, old somatic dimensions may be better appreciated.

Yes, Pressman employed the approaches of his many predecessors, as Dwyer comments. Yet now it is possible to see the significance of his work in the context of the long history of the writing of the history of psychiatry. He has shown that it is not necessary to depict the prepsychiatric era in a wholly negative way. Nor is it necessary to portray the institutional stage in unfavorable terms, nor even to attack the environmentalists, or to focus on the disastrous consequences of deinstitutionalization of the mentally ill. Historians should never again be captive to unbalanced and partisan negative readings of the past.

Pressman in this way showed the future when he used the new eclecticism to move to a positive narrative, however nuanced and layered. I believe that his new positive discourse will be paradigmatic for historians of psychiatry for many years, as they tell the story of humans' struggles with types of afflictions that have been described in the West for twenty-five centuries.

22. I believe that a strong foreshadowing of this development can also be found in the work of Gerald Grob.

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